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Correct Answer

Next Q

Practice Exam - 1

Question 48 of 117



Case: A 49-year-old male with a history of Human Immunodeficiency Virus (HIV), CKD IIIa (baseline serum creatinine (SCr) of 1.5), hypothyroidism, non-ischemic cardiomyopathy (LVEF 30%), s/p CRT-D, admitted to an outside hospital following a device shock, found to be in acute decompensated heart failure receiving intravenous diuretics, course is complicated by acute kidney injury (SCr 2.3, K 5.3) and asymptomatic hypotension (85/50 mmHg). On device interrogation, he had ventricular tachycardia. He is now transferred to your facility out of concern for inability to restart guideline directed medical therapy (GDMT) including Valsartan and spironolactone.

As you look through his records, you learn that he has been admitted to your facility twice this year prior to this admission, and his cardiologist held his sacubitril/ valsartan due to lower blood pressures three months prior to this admission. He also tells you that prior to his device shock, he was only comfortable at rest and was noticeably short of breath during regular activities even at home. His SCr is 2.5 today.

What is your next course of action?

- A. You continue his beta blocker and ask physical therapy to ambulate him more frequently
- ☒ B. You launch advanced therapies evaluation
- C. You explain to your team that heart transplant is not an option given his HIV status
- D. You rechallenge him with sacubitril/ valsartan
- E. You restart his spironolactone and monitor his potassium levels

Commentary

References

Correct Answer: B. You launch advanced therapies evaluation

Teaching Point: AHFTC should know criteria and appropriate timing for advanced therapies evaluation.

Rationale: Patient meets several criteria of advanced heart failure, including NYHA IIIb, end-organ dysfunction, defibrillator shocks, hospitalizations, low blood pressure, and inability to tolerate GDMT.

Incorrect Answers:

A. No data to support that more frequent ambulation will reverse the patient's acute conditions.

C. Heart Transplantation is not contraindicated in patients with HIV.

D & E. HF guidelines recommend caution with the use of sacubitril/ valsartan and spironolactone with low systemic blood pressure, renal insufficiency and a potassium level of >5 , especially as his SCr continues to rise.

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